A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Peter Holsman
NIIM Clinic

Dear Dr Holsman,

Re: Ms Sally Cziesla
DOB: 23/05/1978 Phone: 0438608294
Address: 2549 Ballan-Daylesford Road, DAYLESFORD, VIC, 3460
Email: salcziesla@gmail.com

Summary

- **Iron deficiency anaemia with a ferritin of 16 µg/L.**
- **Small bowel erosions with normal faecal calprotectin, unlikely to represent Crohn's disease.**
- **Functional gastrointestinal symptoms and associated temporomandibular joint (TMJ) dysfunction.**
- **Commencing low-dose nortriptyline.**
- **Action required: Please arrange an iron infusion and review iron studies in four months.**

I had the pleasure of conducting a telehealth review with Sally. We primarily discussed her recent investigation results and her ongoing symptoms of lethargy and gastrointestinal distress.

Sally's iron levels have shown a progressive decline. Her ferritin was 103 µg/L in October last year, dropped to 38 µg/L in February, and is now 16 µg/L. This explains her recent feelings of being "flat". It is notable that this decline has occurred despite her previous iron infusion in August. Interestingly, her faecal calprotectin levels have remained low and normal, which argues against active inflammation or Crohn's disease being the primary driver of her symptoms. Furthermore, her previous trial of budesonide did not yield any significant clinical improvement or stabilisation of her iron levels.

I suspect her iron deficiency is multifactorial, likely resulting from a combination of her menstrual cycle and the small intestinal erosions we have previously identified. While I have considered rarer aetiologies for these erosions, such as Behcet's disease, she lacks the typical oral or vaginal ulceration, and her Northern Eastern European ancestry makes this diagnosis statistically unlikely. At this stage, I do not believe she has Crohn's disease, given the normal inflammatory markers and previous biopsy results.

We also discussed her persistent gut symptoms. I suspect these are functional in nature and perhaps distinct from the erosions. Sally also continues to struggle with significant TMJ issues and teeth clenching. This constellation of symptoms suggests a degree of autonomic gut dysfunction and visceral hypersensitivity.

Impression and Plan I have recommended that Sally undergo a repeat iron infusion to address her current deficiency and improve her quality of life. She has expressed a preference to have this arranged with you locally.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Regarding her functional symptoms and TMJ dysfunction, I have suggested a trial of low-dose nortriptyline. I have explained that we are using this for its effect on the gut-brain axis rather than for any neuromodulatory effect on mood. I have started her on 10 mg at night, with a view to increasing this to 20 mg if tolerated. I have counselled her regarding potential side effects, specifically nocturnal grogginess and dry mouth.

I intend to monitor her progress closely and will review her in four months with repeat iron studies. If her iron requirements remain high, we may consider a repeat capsule endoscopy next year to ensure there is no progression of her small bowel erosions.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Sandeep Dhaliwal

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